

From : I.Mokalake@medicalschemes.co.za

To : "info@ethichawks.co.za"<info@ethichawks.co.za>

Cc : "Complaints"<complaints@medicalschemes.co.za>

Subject : FW: URGENT: Joint Ethical Ruling – 48-Hour Response Required – Exhausted Referral Chain (CMS → HPCSA → SANC → SAPC) – Imminent Life Threat [GEMS 000655789]

Date : 07 May 2026 12:01

Dear Mr Mathee

Thank you for contacting the Council for Medical Schemes and please accept our apology for the delayed response.

1. For our office to investigate your matter, it must fall within our jurisdiction. Section 1 of the Medical Schemes Act (the MSA) defines a complaint as “*a complaint against any person required to be registered or accredited in terms of this Act, or any person whose professional activities are regulated by this Act, and alleging that such person has—*
 - *acted, or failed to act, in contravention of this Act; or*
 - *acted improperly in relation to any matter which falls within the jurisdiction of the Council”.*
2. Therefore, for us to investigate your complaint, the entity complained against must first be an entity registered or accredited in terms of the MSA, **and** you must allege, with supporting evidence, that the said entity contravened a **specific provision of the Act**. The mere fact that an entity is registered with us is not sufficient; you must demonstrate or allege that a provision of the Act was contravened (not labour related issues).
3. While we note that there is a history to your matters and issues, some of which may fall outside our jurisdiction, you are requested to complete the attached complaint form and clearly indicate only those facts which you believe fall within our jurisdiction, as well as the recourse you seek. This should include the identity of the parties complained against, what they are alleged to have done, which provision(s) of the Medical Schemes Act were allegedly contravened, and the relief sought. Please ensure that you also submit evidence in support of the allegations. A copy of the complaint form is attached for ease of reference (an additional page may be attached if the space provided is not sufficient).
4. The matter will be considered upon receipt of the documents
5. We also take note of Section 4 of your email below and wish to comment that, while it is common knowledge that any contravention of legislation and/or unlawful activities should be reported to the relevant authorities, we are not in a position to issue formal statements on matters not fully interrogated by our office.

Kind Regards,

Lebogang Mokalake
Senior Legal Adjudication Officer
Council for Medical Schemes
t:
e: l.mokalake@medicalschemes.co.za
[Disclaimer](#)

From: Wilbur William Matthee <info@ethichawks.co.za>
Sent: Tuesday, 14 April 2026 13:25
To: professionalpractice@hpcsa.co.za; Complaints <complaints@medicalschemes.co.za>;
registrar@sanc.co.za; registrar@sapc.za.org
Cc: Professional.Affairs@sapc.za.org; complaints@sahrc.org.za
Subject: URGENT: Joint Ethical Ruling – 48-Hour Response Required – Exhausted Referral Chain
(CMS → HPCSA → SANC → SAPC) – Imminent Life Threat [GEMS 000655789]

To the Registrars and Executive Authorities of South Africa's Health Regulators,

I am Sr. Wilbur W Matthee, a Specialist Registered Nurse (BTech; PG Dip HSM, NQF 8) and Founder of EthicHawks Forensic Governance.

SECTION 1: THE REFERRAL CHAIN – JURISDICTIONAL PING-PONG

At the start of 2026, I lodged my complaint with the Council for Medical Schemes (CMS) . CMS declined jurisdiction and referred me to the Health Professions Council of South Africa (HPCSA) . HPCSA then referred me to the South African Nursing Council (SANC) . SANC has failed to acknowledge or investigate my complaint. I have now also been engaged with the South African Pharmacy Council (SAPC) , where the Professional Conduct Committee showed hesitation in ruling on the fundamental ethical question below.

I have exhausted every referral path. No regulator has taken ownership. This jurisdictional ping-pong is a classic tactic that delays justice while regulators avoid responsibility. I am now addressing all four Councils simultaneously because this matter transcends any single jurisdiction.

SECTION 2: NOTICE OF REGULATORY FAILURE (SANC)

At the start of 2026, I filed a formal professional misconduct complaint with SANC. To date, SANC has failed to acknowledge or investigate this report, which involves a Priority 1 systemic risk. I am now escalating this as a matter of institutional bad faith.

During a meeting with the SAPC Professional Conduct Committee on 14 April 2026, the Committee showed hesitation in ruling on a fundamental ethical question. I am now putting that question to all of you:

"Does the statutory duty of a registered professional to prioritize patient safety and the public interest permit the refusal of an employer's instruction when that instruction is based on verified, flawed forensic data intended to suppress a 7-year acute financial drainage issue?"

SECTION 3: BACKGROUND

- Error: I identified a 7-year siphoning of funds affecting a 75-year-old widow.
- Professional Negligence: I was instructed to "probe" the member for non-adherence using data I knew to be forensically flawed.
- Retaliation: I was charged with insubordination for refusing to execute a professionally negligent act. Upon making a Protected Disclosure (GRC Memorandum, 16 Dec 2025), I was accused of being impatient and unreasonable.

SECTION 4: FORMAL REQUEST

I request a joint or individual statement of position on whether:

- Professional registration is subordinate to corporate operational convenience.
- A specialist is ethically required to report systemic siphoning of funds, or if "corporate secrecy" overrides the Nursing Act, Pharmacy Act, Prevention and Combating of Corrupt Activities Act, Consumer Protection Act, Health Professions Act, National Health Act, Medical Schemes Act, and Protected Disclosures Act.

SECTION 5: URGENT ADDENDUM – IMMINENT LIFE THREAT

This serves as formal notice that due to the retaliatory action taken against me following my Protected Disclosure, I currently lack funds for food or accommodation. My life, dignity, and constitutional rights to access to food and housing (Section 27 of the Constitution) has been violated.

I hereby request a decision from your office within 48 hours of receipt of this notice.

If no decision is received by close of business on Thursday, 16 April 2026, I will have no choice but to approach the Public Protector and the High Court on an urgent basis to compel your response, citing:

- Violation of my constitutional rights to administrative justice (Section 33 of the Constitution);
- Regulatory failure to uphold statutory mandates;

· Retaliation against a protected whistleblower under the Protected Disclosures Act.

SECTION 6: DELAY AS CONTINUING OCCUPATIONAL DETRIMENT

I further submit that the collective failure of the Respondent Regulators to issue a ruling or acknowledge my protected disclosure constitutes an independent and ongoing occupational detriment as defined in Section 1 of the Protected Disclosures Act 26 of 2000.

The excessive and unexplained delay violates the constitutional standard of "prompt justice" as affirmed in *Mapyane v South African Police Service*. The prejudice I am suffering includes:

1. Prolonged uncertainty impacting my ability to seek alternative employment or obtain professional references, thereby perpetuating my destitution.
2. Destitution directly resulting from retaliation – my current lack of food and accommodation flows directly from the employer's retaliation following my protected disclosure. The regulators' silence validates that retaliation.
3. Regulatory failure amounting to maladministration – by refusing to act on a Priority 1 systemic risk, the Councils are allowing a corporate entity to silence a whistleblower through attrition, which constitutes a breach of their statutory mandates.
4. A chilling effect on all health professionals – by failing to rule on this matter, the Councils are sending a clear message that whistleblowing leads to destitution and regulatory silence.
5. Exhaustion of remedies – I have followed every referral path (CMS → HPCSA → SANC → SAPC) and am now at the end of the line. There is no further internal avenue available to me.

SECTION 7: THE QUESTION BEFORE YOU

I put it directly to each Registrar:

Is the delay you are collectively imposing a form of constructive denial of justice?

If each Council believes the other has jurisdiction, then none of you are fulfilling your statutory mandates. I require a clear, written answer within 48 hours.

SECTION 8: ESCALATION NOTICE

The evidence attached does not require a 3-year inquiry. I require a response in 48 hours.

In the absence of a response by close of business on 16 April 2026, I will simultaneously:

1. Proceed to the Public Protector to report a collective failure of healthcare regulators to uphold their statutory mandates;

2. File an urgent complaint with the South African Human Rights Commission (SAHRC) for violations of Sections 26, 27, and 33 of the Constitution;
3. Approach the High Court on an urgent basis to compel your response, citing the jurisdictional ping-pong as evidence of bad faith.

Your silence will be recorded and submitted as evidence of regulatory failure.

LIST OF ATTACHMENTS (Annexures):

Annexure A: VICT1 – Email chain (Oct 2025) showing: (i) investigation finding system error; (ii) instruction to use flawed data; (iii) ethical refusal

Annexure B: GRC3 – Maximum Severity GRC Escalation (16 Dec 2025) including reference to audio recording of retaliation

Annexure C: IMP1 – Impact Statement documenting hostile work environment and occupational detriment

Annexure D: CRO1 – Email from Chief Risk Officer instructing me to stop communicating (institutional silencing)

Annexure E: GRC2 – Employer's Opposing Affidavit (10 Dec 2025) containing character attacks

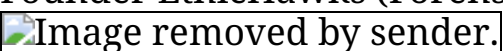
Annexure F: RES1 – Formal Notice of Resignation citing automatically unfair constructive dismissal

Annexure G: MED1 – Medical report confirming severe depression and PTSD due to retaliation

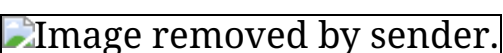
Annexure H: EXIT3 – Certificate of Service misrepresentation and UIF non-compliance (Jan 2026)

Yours professionally,

Sr. Wilbur W Matthee
Specialist Registered Nurse, BTech (Psych, Midw,Comm), PG Dip HSM (NQF 8)
Health Services Management.
Founder EthicHawks (Forensic Governance and Compliance)

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www.ethichawks.co.za

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